

SECURE PATIENT PORTAL MESSAGE

From: Dr. Rachel Green, Leukemia Service

To: Dr. Michael Torres, Endocrinology

Date: March 18, 2024

Subject: Barbara White - Glycemic Management on Enasidenib

Hi Mike,

Thanks for agreeing to co-manage Barbara White (MRN AML014, DOB 3/29/51).
Quick rundown:

Diagnosed with intermediate-risk AML with IDH2 mutation (R172K) in mid May 2023. Given her age (72), mild CHF (EF 45%), and patient preference, we went with venetoclax 400mg + enasidenib 100mg daily rather than intensive chemo. She started on May 23, 2023.

She's doing GREAT - achieved CR after 2 cycles and maintaining MRD-negative remission. The issue: enasidenib is causing significant hyperglycemia (IDH inhibitor syndrome). Pre-treatment A1c was 5.8%, now 8.9%.

Current sugars 200-350 despite metformin 1000mg BID and glipizide 10mg BID. She's checking sugars QID, following diabetic diet. No prior DM history. BMI 24.

Can't stop enasidenib - it's keeping her leukemia controlled. Venetoclax dose already reduced to 200mg for cytopenias.

Could you see her this week? May need insulin. Also, FYI - enasidenib can cause differentiation syndrome, so if she develops dyspnea/fever/edema, call me immediately (not DKA).

Recent labs (3/15/24):

- Glucose 287
- Cr 0.9, K 4.2
- WBC 3.1, Hgb 10.8, Plt 95K (expected on therapy)
- LFTs normal

Thanks! Rachel

P.S. - She's delightful, retired teacher, very compliant. Grandson getting married in June - she's determined to make it!